Your name (recorded on each decision)

NP on duty

Show cases whose current outcome is

All

Criteria

- Is type 2 diabetes documented as a diagnosis?
- Is the member 18 or older?
- Is the most recent HbA1c 7.0% or higher?

AI confidence floor: **0.9**

Prior authorization review

Policy: GLP-1 receptor agonist for type 2 diabetes (version 2026-11-11). The system approves clear cases; everything else waits for a reviewer. Every action is appended to the paper trail.

Queue (35)

approve: 17 · unclear: 10 · deny: 7 · request_more_info: 1

	request_id	current_outcome	AI confidence	dru
☐	PA2022000032	approve	0.98	Oze
☐	PA2023000101	approve	0.98	Oze
☐	PA2023000176	request_more_info	0.98	Trul
☐	PA2025000435	approve	0.98	Trul
☐	PA2025000523	approve	0.98	Mou
☐	PA2022000023	approve	0.97	Oze
☐	PA2022000069	unclear	0.97	Oze
☑	PA2023000132	unclear	0.97	Oze
☐	PA2023000161	deny	0.97	Oze
☐	PA2023000208	approve	0.97	Mou

PA2023000132 — Ozempic 1 mg/dose pen

Member **MBR2023003496** · requested 2023-07-23 · prescriber Endocrinology

Current outcome: **unclear** (by system)

AI assessment — Near threshold: Is the most recent HbA1c 7.0% or higher? (7.0) (confidence 0.97)

	Criterion	Value	Quote from note
✅	Is type 2 diabetes documented as a diagnosis?	True	Type 2 diabetes mellitus, not at goal.
✅	Is the member 18 or older?	60	DOB:** 12/17/1962 (60-year-old female)
✅	Is the most recent HbA1c 7.0% or higher?	7.0	HbA1c: 7.0% (drawn 07/16/2023)

Clinical note

Endocrinology Progress Note

Patient: Stephanie Smith
DOB: 12/17/1962 (60-year-old female)
Date of Service: 07/23/2023
Clinician: Alan Coleman, MD, Endocrinology

HPI:
Ms. Smith is a 60-year-old woman with type 2 diabetes mellitus who presents for follow-up of glycemic control. She has been taking metformin for about 6 months and reports good adherence. She is tolerating metformin well, with no GI upset, diarrhea, or other adverse effects, and has no contraindication to continuing it. Despite consistent adherence and ongoing dietary efforts, her glycemic control remains above her individualized goal. She denies polyuria, polydipsia, blurred vision, or symptomatic hypoglycemia. She would like to add an agent that may also support weight management.

Past Medical History:
- Type 2 diabetes mellitus
- Hypertension, controlled
- Hyperlipidemia
- Obesity

Family/Personal History:
She has no personal or family history of medullary thyroid carcinoma or multiple endocrine neoplasia type 2 (MEN2). She has no known history of pancreatitis.

Medications:
- Metformin 1000 mg PO twice daily (ongoing, about 6 months)
- Lisinopril 10 mg PO daily
- Atorvastatin 20 mg PO daily

She is not currently taking any GLP-1 receptor agonist.

Allergies: No known drug allergies.

Vitals:
BP 128/78, HR 74, Temp 98.2°F, Wt 204 lb, BMI 33.9.

Exam:
General: Well-appearing, in no acute distress.
Neck: Thyroid non-enlarged, no palpable nodules.
CV: Regular rate and rhythm, no murmurs.
Lungs: Clear bilaterally.
Extremities: No edema. Monofilament sensation intact bilaterally.

Labs:
- HbA1c: 7.0% (drawn 07/16/2023)
- Renal function within normal limits on recent panel.

Assessment:
1. Type 2 diabetes mellitus, not at goal. HbA1c is 7.0% despite about

6 months of metformin therapy, which she tolerates well. Adding a GLP-1 receptor agonist is appropriate for further glycemic improvement and weight benefit. She has no personal or family history of MTC or MEN2 and is not on any other GLP-1 agent.
2. Hypertension, controlled.
3. Hyperlipidemia, on statin.

Plan:
- Continue metformin 1000 mg twice daily.
- Add Ozempic (semaglutide) 1 mg/dose pen, subcutaneous once weekly.
- Counseled on common GI side effects, hydration, and the signs and symptoms of pancreatitis and gallbladder disease. Reviewed the boxed warning regarding thyroid C-cell tumors.
- Continue lifestyle modification, with referral to diabetes education.
- Repeat HbA1c in 3 months.
- Follow up in clinic in 3 months, sooner as needed.

I am requesting Ozempic 1 mg/dose pen for this patient to improve glycemic control of her type 2 diabetes.

Alan Coleman, MD
Endocrinology

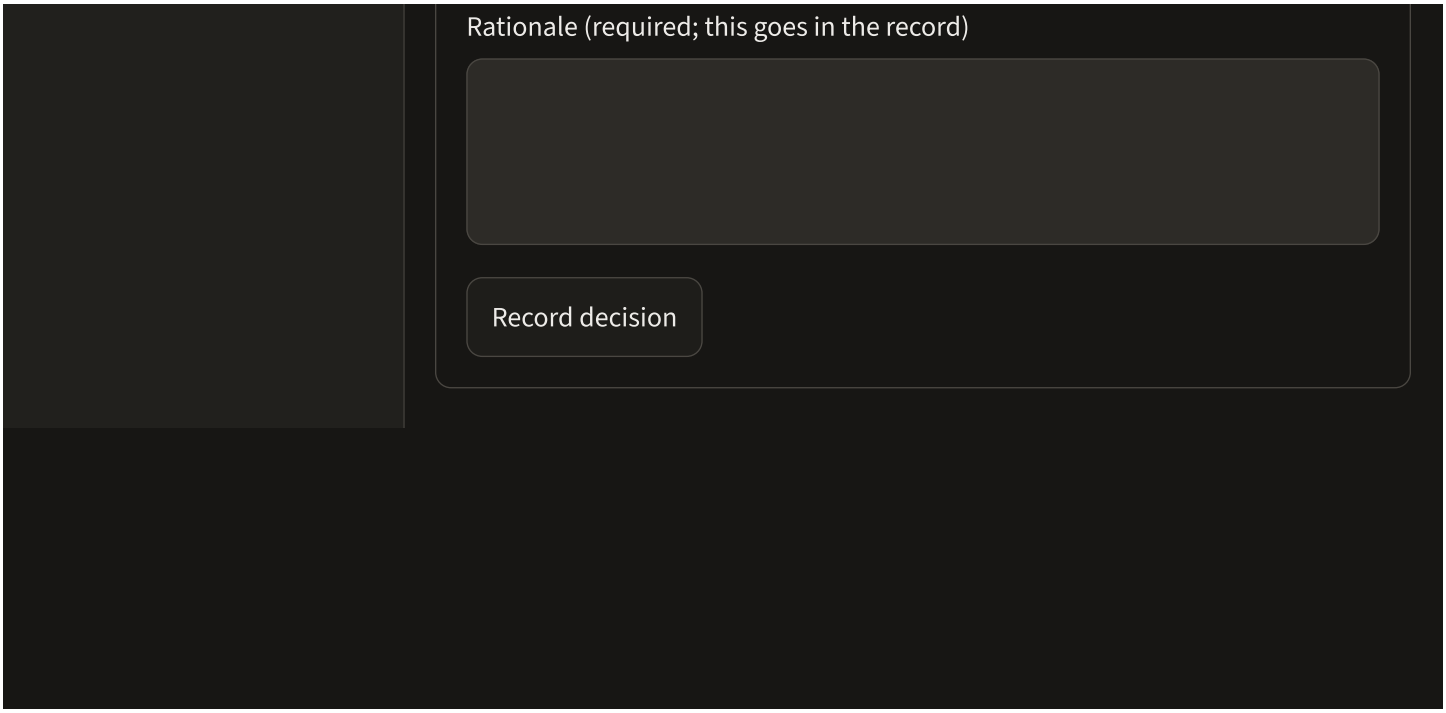
History

decided_at	decided_by	outcome	reason
2026-11-12 00:24:14+00:00	system	unclear	Near th

Your decision

Outcome

approve deny request more info


Rationale (required; this goes in the record)
Record decision